SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cisplatin every 3 weeks + head and neck radiotherapy

Locally advanced stage III–IV squamous head and neck cancer. · NCCP 00387 · version 5 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Cisplatin and radiotherapy work together to treat locally advanced head and neck cancer. Your team explains the treatment aim, likely benefit and alternatives for your diagnosis.

Schedule
Cisplatin 100 mg/m² on Day 1 every 21 days for three planned cycles during radiotherapy. Cisplatin is given before radiation on the same day. If radiotherapy is cancelled, cisplatin is held until radiation resumes; the team updates both calendars.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Nausea and vomiting
- ☐ Kidney injury and mineral changes
- ☐ Hearing loss or tinnitus, which can persist
- ☐ Nerve damage, which can persist
- ☐ Infusion reaction and effects on fertility

Radiotherapy
LOCAL TREATMENT

- ☐ Sore mouth and throat
- ☐ Dry mouth, taste and saliva changes
- ☐ Swallowing or voice problems
- ☐ Skin reaction in the treated area
- ☐ Jaw stiffness and dental problems
- ☐ Some swallowing, saliva or tissue effects can be lasting

Discuss the individual aim, alternatives and uncertainty; serious or lasting harms; treatment burden; fertility and pregnancy precautions; and the patient's priorities. Explain blood-count suppression/infection, nausea, kidney/mineral injury, hearing loss and neuropathy that can persist, hydration and same-day radiation coordination. Discuss radiation mouth, skin, swallowing, saliva, dental and jaw effects, including lasting effects and potential nutritional support. Radiation has its own consent process. Confirm actual urgent contacts and teach-back; this draft does not authorise treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00387 v5.

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